

CONFIDENTIAL NEUROPSYCHOLOGICAL ASSESSMENT REPORT

Client Name: Barran MCLEAN
Date of Birth: 08/04/1973
UR Number: 2645287
Date of Assessment: 05/01/2023
Date of Report: 13/01/2023

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BACKGROUND INFORMATION

Reason for Referral

Barran 'Richard' McLean is a 49-year-old male who was admitted to Clare Moore Building (CMB) on 19/11/2022 in the setting of a psychotic relapse with prominent persecutory delusions. He has been on a Compulsory Treatment Order (CTO) and his community team was varied from CTO to Inpatient Treatment Order (ITO) due to poor engagement and evasion in community. He was referred for a neuropsychological assessment by Dr Thomas Carlyon-Stewart (HMO, CMB) for characterisation of his cognitive profile and for diagnostic clarification about the **presence of a neurodegenerative process**.

Medical History

Past medical history includes gastro-oesophageal reflux disease (GORD), syphilis (treated with benzylpenicillin during the current admission) and psoriasis. He is a current cigarette smoker. Richard reported frequent methamphetamine use in the community over the past several years, although it remains unclear as to the amount and frequency of his use, into which he showed very limited insight into its impact on his mental health. He was previously prescribed dexamphetamine (for ADHD) but this was weaned during the admission earlier this year due to prominent affective symptoms with ongoing psychotic symptoms and methamphetamine use in the community. With regard to his use of other substances, Richard reported previous use of ecstasy and cannabis. He also consumes alcohol. Current medications include paliperidone depot 150mg monthly, amisulpride 600mg nocte and diazepam 2mg BD PRN. His CT (dated 22/12/2021) and MRI brain imaging (dated 04/01/2023) were both normal. He also reported a family history of dementia on the maternal side including his grandmother and aunt. Of note, he reported that they had developed memory problems in their 70-80s, however he was unsure of their formal diagnoses.

Psychiatric History

Richard has a history of schizophrenia (diagnosed in 1994), ADHD and depression. He was previously managed by private psychiatrist, psychologist and GP over many years. He also reported being sexually abused by a family friend during his teen years.

Richard has had a number of psychiatric inpatient admissions to optimise his mental health, reduce the risk of self-neglect and vulnerabilities, prevent serious risks to himself and others, and attempt to reduce

contact with a number of agencies. Below is a summary of Richard's inpatient admissions and community follow-up/case management:

- 23/08/2020 – 22/09/2020: Richard's GP, Dr Simon Cooper, made a referral to Saltwater Community Mental Health Team (SWCMHT) for case management and a psychiatric review in the setting of escalating distress and related concerns due to his need to "self-medicate". As per the GP's referral letter, Richard's accommodation at the time was under threat, he was financially "strapped" and his work as an NDIS support worker was piecemeal. Upon engagement with SWCMHT, Richard denied the diagnosis of schizophrenia although he agreed with it in the past. The main stressors at the time were related to his two-year long legal battle against his previous GP to whom Richard had voiced suicidal ideation to, who proceeded to prescribe medication on which Richard believed he could have overdosed on, COVID lockdowns, isolation, limited social supports and strained relationship with his family.
- 25/02/2021 – 04/03/2021: Richard was admitted to CMB under Temporary Treatment Order (TTO) for relapse of paranoid schizophrenia. Of note, this was his first inpatient admission. Initially Richard was referred to CATT by his friend who voiced concerns about his deteriorating mental health and had plans to commit suicide. Additionally, his friend reportedly expressed concern about Richard's focus on his website was leading to increasing levels of distress, and had listed suicide dates and "increased venom towards former GP and healthcare professionals". He had a significant suicide attempt via cutting of left cubital fossa on 25/02/2021 with parts from a vape pen. He reported attempting suicide with intent to die due to having his dexamphetamine ceased on this admission, not feeling appropriately listened to by staff, and being on a TTO. This resulted to his transfer to the ICU for eight days.
- 09/03/2021 – 17/09/2021: Following his discharge from CMB on 09/03/2021 as a voluntary client Richard was referred for case management under the SWCMHT. Of note, he was reportedly fixated on receiving dexamphetamine scripts and he felt like case management and psychology had nothing to offer him. Soon after discharge, he disengaged with the SWCMHT and he was discharged from case management in September 2021.
- 21/12/2021 – 24/12/2021: Richard was initially assessed by CATT and he was subsequently admitted to CMB involuntarily under TTO for methamphetamine-induced psychosis on the background of schizophrenia, amphetamine dependence and ADHD. He was admitted as an involuntary patient to the CMB with psychotic symptoms. He was discharged as voluntary and referred to Community Plus team for case management.
- 04/01/2022 – 11/01/2022: Richard was re-referred for case management with Community Plus team in early January 2022. The psychosocial stressors at the time included falling behind in rent payments and issues with NDIS. His primary concern was related to complex delusional system involving government agencies conspiring against him. Richard refused to engage with Community Plus team and multiple home visits were attempted with very limited success. He was discharged with CATT support and reportedly returned to methamphetamine use shortly after discharge.
- 29/03/2022: Richard was placed on assessment order in early March 2022 in the setting of concerns regarding suicidality, acute psychosis and ongoing persecutory delusions. At the time, he was living on his own with his dog, experiencing financial hardship as he was unable to pay rent for the past three months, and struggling to afford groceries. He was discharged as a voluntary patient after assessment at ED. He was re-placed under assessment order at the end of March, resulting in brief admission to CMB in setting of psychotic symptoms (chronic delusions documented as stating being persecuted by the CEO of the hospital and that there was a conspiracy against him, paranoia and tangentiality). He was discharged as a voluntary patient 24 hours later with opinion that majority of issues likely secondary to amphetamine abuse and psychosocial stresses. At the time, ongoing Community Plus engagement was recommended, however Richard had ongoing difficulties engaging despite assertive efforts.

- 08/06/2022 – 29/06/2022: Richard was admitted to CMB and commenced on paliperidone depot. He was placed on an Assessment Order on 07/06/2022. At the time, he presented with a high-level of delusional content relating to being the target of elaborate conspiracy by all levels of government. There was also a significant decline in his functioning over an extended period of time in relation to this delusional content, being determined to be at considerable risk of reputational damage. He spent the entirety of his admission in ICA. Richard found this admission very frustrating whereby he assaulted two staff member and had a physical altercation with a co-patient. These behaviours of concern resulted in multiple Code Greys. On 27/06/2022, Richard completed an MMSE and the score was 29/30 (gave the wrong date).
- October 2022: Richard was taken to the ED on two occasions (18/10/2022 and 20/10/2022) in the context of worsening psychotic symptoms, escalation in agitation and a clear increase in voicing threats towards himself and others. Of note, he admitted to returning to methamphetamine use after returning home from previous admission in June 2022. On both occasions Richard absconded from the ED.
- 19/11/2022 – 11/01/2023: Richard was admitted to CMB after his CTO was varied to an ITO. This occurred in the setting of persistent symptoms of psychosis in the community including highly systematised and entrenched chronic persecutory delusions and persistent disengagement and evasion of community psychiatric treatment over many months. Based on file notes, his memory appeared to be significantly impaired during this admission. In particular, he would forget small details and is frequently unable to remember clinicians and previous discussions. He completed a cognitive screening with Tamar Ben-Barak, Occupational Therapist, on 22/12/2022. His Montreal Cognitive Assessment (MoCA) was 26/30. According to the OT progress note, his total MoCA score fell within the normal range. Qualitatively, Tamar noted that Richard had difficulties with visuo-spatial, memory and attention-related tasks. Of note, his verbal fluency was just within normal range (11 words per minute, which was the minimum cut-off score). Upon discharge, the principal diagnosis made was of a psychotic relapse of schizoaffective disorder with prominent entrenched chronic persecutory delusions.

Forensic History

Richard does not have any known forensic history, however he has made threats towards family and staff (this has been a reason for restricting phone access during his current admission). He has a history of physically assaulted a consultant (e.g. taking spectacles off his face and crushing them into pieces before throwing frame at wall) and a nurse (e.g. throwing a cup of water over her) during the June 2022 admission. He was apologetic for his behaviours at the time. His parents have previously had an intervention order (IVO) instated against Richard.

Psychosocial History

By way of brief psychosocial background, Richard was born and raised in Australia. He is the middle child of a sibship of three (an elder sister and a younger brother). He is well educated and he has completed both a Master's degree in Education and a PhD in Ethics at Victoria University (Melbourne). He wrote an autobiography about his experience with schizophrenia. Professionally, he previously worked as a consumer consultant, mental health advocate with SANE Australia, The Mental Health Research Institute and The Dax collection and others. He formerly worked in the NDIS in a patient support role. He is a recipient of NDIS and has an allocated NDIS worker. Prior to his current admission, he was residing in a private rental house in Footscray. He had lost his job as an art therapist with NDIS due to his mental state deterioration. He has been under financial issues for several years.

Richard is single with no children and he has been engaged twice. He has very limited social supports. He has a strained relationship with his family due to significant carer stress. He speaks to his parents occasionally and has no contact with his siblings. He enjoys spending time with his dog, Crystal.

CURRENT ASSESSMENT

Self-report

On clinical interview, Richard reported a number of cognitive changes since his suicide attempt in February 2021. In particular, he stated his memory had "dropped" (i.e. significant difficulty with learning and remembering new information) and added that his memory difficulties had become "worse" over time. He stated that prompts and cues do not assist with his recollection of the previously learnt information, explaining that the reminders as "scribbles" to him. On questioning, Richard described his attention as "bad". More specifically, he described he is not able to "concentrate for a long time". In addition, he felt his thinking speed had become "slower" as if his "brain is rusty". He also described experiencing word-finding difficulties and felt these have been "steadily worse". Further, he noted some changes in his executive functions including disorganisation, inability to make decisions and difficulties with problem solving skills.

When asked about his mood, Richard stated he was feeling "depressed", citing that he had "nowhere to go to", "no agency" and that he was "scared of being killed by his former partner". He added that his main concerns were related to his "financial struggles" in the community. Additionally, he noticed his sleep has been affected by his mental health.

With regard to his activities of daily living, Richard reported he has "no money" and as a result t he has been "squatting" with "no food" and "no cleanliness". He described his house as a "dump" and he felt "hopeless". He also noted that he had stopped working since He added that he has trouble using his new mobile phone (i.e. Android) and previously had an iPhone. He commented that he is more familiar with Apple devices.

Collateral history

Collateral history/information was not obtained as Richard did not provide verbal consent for clinician to contact his family members or friends.

Presentation: Richard was seen over several sessions in a single day with multiple short breaks. He presented as a polite and cooperative man. Richard described his mood as "depressed" and he was teary when discussing his past history of trauma. Affect was blunted and restricted in range. Behaviour was generally appropriate, however reduced frustration tolerance with underlying irritability was noted throughout the assessment. Speech was fluent and devoid of word-finding difficulties. He was markedly tangential and perseverative in conversation, frequently answering interview questions by talking about litigation issues against the hospital and related matters. Thought content was mainly related to his frustration at the healthcare system in Australia and his unjust treatment. Richard denied experiencing hallucinations or paranoia, and he did not appear to be responding to internal stimuli. He also denied having any suicidal ideations, plan or intent. Richard appeared to be putting in adequate efforts throughout testing and his performance did not appear to be affected by fatigue or pain. The current assessment was therefore deemed to be an accurate reflection of his current cognitive functioning. Insight into his current cognitive functions was superficially intact, however insight into his mental health and the reason for current admission was poor.

Tests Administered:

Test of Premorbid Functioning (TOPF); Wechsler Adult Intelligence Scale, Fourth Edition (WAIS-IV) – selected subtests; Wechsler Memory Scale, Fourth Edition (WMS-IV) – selected subtests; Hopkins Verbal Learning Test, Revised Edition (HVLT-R); Trail Making Test; Rey Complex Figure Test; Clock Drawing Test; Simple Copy; Sydney Language Battery (SYDBAT) – Naming subtest; Victoria Stroop Test; mini-Social cognition & Emotional Assessment (mini-SEA); Emotional Recognition subtest; and Depression, Anxiety and Stress Scale, 21-item version (DASS21).

Results of Assessment: Where appropriate, test results will be reported using the following descriptions: "Very Superior" (98th percentile and above), "Superior" (91st to 97th percentile), "High Average" (75th to 90th percentile), "Average" (25th to 74th percentile), "Low Average" (9th to 24th percentile), "Borderline" (2nd to 8th percentile) and "Extremely Low" (<2nd percentile). A percentile indicates a person's ranking between 0 and 100 relative to other people, so that, e.g., if a score falls at the 80th percentile, the person has performed better on that task than 80 people out of 100. Unless stated otherwise, these comparisons are made with reference to other people the same age.

Premorbid Functioning: Based on educational and occupational attainment, as well as performance on tasks deemed to be less vulnerable to the effects of neurological change, the estimate of pre-morbid intellectual functioning placed Richard between the Average and the High Average range.

Orientation: Richard was well oriented to person, place and time. He could accurately recall the current Prime Minister of Australia and describe a recent news items, although he could not recall the names of the former Prime Minister of Australia and the current President of the United States of America.

Information Processing Speed: Richard's information processing speed was variable, with task performance ranging from intact to **moderately slowed**. Psychomotor speed was mildly reduced.

Attention: Immediate auditory attention span was within normal limits, with task performance falling within the Average range. Working memory (i.e. the ability to hold information in mind and manipulate it for further use) was intact. More specifically, Richard was able to recite a string up to five digits forward reliably and seven unreliably, four digits backward reliably and five digits unreliably, and he could sequence a five-digit string reliably.

Memory: New learning of auditory-verbal information (presented in a word list) **was moderately reduced**. Delayed recall of such information was also moderately reduced. New learning and delayed recall of more meaningful information (presented in the format of short stories) was mildly-moderately reduced. Qualitatively, Richard did seem overwhelmed with large amounts of auditory-verbal information. His delayed recognition trials on this task were **mildly reduced**, with **task performance falling within the Low Average range**. His ability to learn and remember visual materials (in the format of geometric designs) was intact.

Language Skills: Confrontation naming was intact. Richard's performance on tasks assessing vocabulary and general knowledge was intact (in the Average range). Semantic fluency was within normal limits, although he made some repetition errors.

Visuo-spatial Skills: In relation to visuo-spatial constructional skills, Richard's performance ranged between intact and **severely reduced**. His performance on a task was most likely influenced by planning and organisational difficulties (i.e. an impulsive and piecemeal approach to the task). Qualitatively, there was no primary visuo-spatial impairment noted on testing.

Executive Functions: Performance on executive functioning tasks was variable. In particular, Richard's verbal idea generation was intact, although he made some repetition errors which is suggestive of reduced self-monitoring skills. Set-shifting (i.e. the ability to alternate attention between two tasks) was **moderately reduced**. Of note, the performance on this particular task was most likely influenced by increased cognitive demands and task complexity. **Planning/organisational skills were severely reduced (with his approach to a complex figure copying task characterised by impulsivity, haphazardness and disorganisation).**

Mood: On a self-reported questionnaire, Richard's answers indicated extremely severe levels of depressive and anxiety symptoms, and moderate level of stress symptoms.

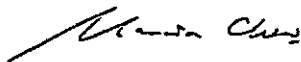
Barran 'Richard' McLean is a 49-year-old male who was admitted to Clare Moore Building (CMB) on 19/11/2022 in the setting of a psychotic relapse with prominent persecutory delusions and a general functional decline over the past several years. He has been on a Compulsory Treatment Order (CTO) and his community team varied from CTO to ITO in the setting of poor engagement and evasion in community. He was referred for neuropsychological assessment by Dr Thomas Carlyon-Stewart (HMO, CMB) for characterisation of his cognitive profile and for diagnostic clarification about the presence of a neurodegenerative process.

There are several factors likely to be affecting Richard's performances on current testing, including his history of schizophrenia, recent psychotic relapse, ADHD, past history of trauma and methamphetamine dependence (over the past 2-3 years). These factors have likely led to a major decline in his mental health, resulting in a high-level of functional disability. There did not appear to be any convincing evidence on current testing to suggest a primary memory disorder or an emerging progressive neurodegenerative process at this point of time. Richard's cognitive difficulties are most likely to be associated with his longstanding history of schizophrenia and his methamphetamine dependence (over the past 2-3 years).

- Due to his extremely poor insight and judgement as well as his compromised cognitive functioning, Richard remains highly vulnerable to exploitation and misadventure. Based on the current assessment findings, an NDIS plan review is strongly supported to advocate for additional funding to assist with Richard's care needs. More specifically, he will need assistance with domestic tasks (cleaning, laundry, shopping and meal preparation), social engagement and community access. His care needs will be most appropriately met in supported accommodation.
- Richard has an existing NDIS plan, however, due to the change of circumstances, I strongly recommend an NDIS plan review meeting to consider and include the following additional supports to his new NDIS plan:
 - Increasing one-to-one support workers: Richard will benefit from having additional hours of support workers who have experience working with people with mental health conditions. This will help him engage in community activities of his interest.
 - Occupational Therapy input (Improved Daily Living funding): Richard requires Improved Daily Living funding for Occupational Therapy to support a successful transition, provide training and support to staff. He will also benefit from having an OT functional capacity assessment.

- Supported Independent Living (SIL) accommodation: Richard will benefit from living in a supported shared accommodation environment. Being supported by trained support workers would help him to live as independently as possible whilst building his skills and capacities.
- Without the further support (listed in the paragraph above) in place, it is very likely that Richard will experience a number of risks related to his mental health. These include:
 - Risk of deterioration in mental health, wellbeing and quality of life: If Richard does not have appropriate supports in place, he is at risk of social isolation and deterioration in his mental health.
 - Risk of substance use: In light of his history of regular substance use (i.e. methamphetamine and Richard can be very vulnerable if he spends more time in the community on his own without adequate supports (i.e. one-to-one support workers).
 - Risk of harm to self: Given his previous suicide attempt and mental health history, Richard would be at risk of harming himself especially when his mental state deteriorates.
 - Risk of progressive further decline in capacity for independence in personal, domestic and community activities
- Ongoing monitoring of Richard's mental state and cognitive functioning is recommended. Should concerns persist, then further investigations would be recommended, including organic screening and neuroimaging, to exclude any possible causes for the observed cognitive difficulties.
- Richard would benefit from an extended period of psychotherapy to improve the management of his schizophrenia, with the aim to returning to his previous level of functioning.
- Given his history of substance use, it is strongly recommended that Richard receive support and counselling. More specifically, he will benefit from having education regarding the impact of substance use on cognition. Richard may benefit from Drug and Alcohol Counselling to support him in ceasing the use of methamphetamine. This will have a long-term effect on his sleep quality, energy levels and mood.
- To support Richard's cognitive difficulties:
 - help to break a task or problem down into component parts and devise a step-by-step plan or structure to follow;
 - written checklists that set out the steps of a task, multiple tasks to be completed, or things to bring to certain events, and that can be 'ticked off' are good, concrete prompts;
 - use simple and direct language;
 - limit the amount of information presented at a time (e.g. keep instructions brief);
 - when a large amount of information needs to be presented or absorbed break it down into more manageable chunks that can be considered independently; and
 - try to assist learning with visual cues such as pictures, diagrams, graphs, maps or flow charts.

A review neuropsychological assessment is not indicated at this point of time. Please do not hesitate to contact me if you require any further information about this report.



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